

30 providers · 4 months on Abridge · 60% utilization · 27,000 documented encounters

THE STORY SO FAR

4 months ago, you deployed Abridge to 30 providers. Since then, those providers have documented **27,000 encounters**—each one generating data about the value Abridge is creating.

The short answer: it is.

This report breaks down exactly where value is coming from, how your results compare to what we typically see, and what the path forward looks like. Every number traces back to your data. Where we've made assumptions, we've flagged them. Where results look unusual, we've called that out too.

The goal isn't to make Abridge look good. It's to give you a picture you can trust—and act on.

\$45K

ANNUAL VALUE CREATED

Defensible hard dollars

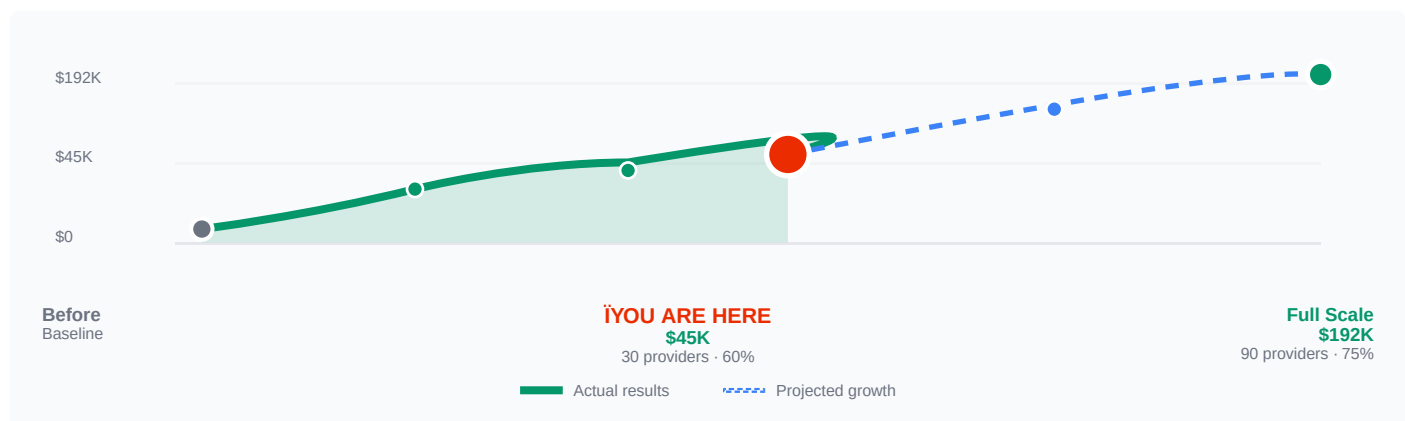
\$1K

VALUE / PROVIDER
Based on 30 providers

+\$148K

EXPANSION POTENTIAL
At 90 providers

YOUR VALUE JOURNEY



VALUE BREAKDOWN

We separate value into three tiers based on how confidently we can measure and attribute it:

Tier 1: Hard Value

Directly measurable financial impact

\$45K

WRVU Per Encounter, 000 encounters x \$33/wRVU x 50% attribution

\$45K

Tier 2: Efficiency Gains

Measured improvements—not yet converted to dollars

Time Saved

2,700 hours/year

Pajama Time Eliminated

-4 hrs/week

Chart Closure Improvement

+10%

We show these in hours and percentages rather than dollars because the conversion varies by organization. If you want to dollarize time savings, we can adjust the model.

WRVU PER ENCOUNTER

Revenue capture improvement

\$45K

annual value

BEFORE ABRIDGE

1.80

wRVU/enc

AFTER ABRIDGE

1.90

wRVU/enc

=

+0.10

6% lift

BENCHMARK

Typical Abridge customers see: 3-7% lift

You're at: 6% — Within typical range

VALUE CALCULATION

 $+0.10 \text{ wRVU/enc} \times 27,000 \text{ encounters} \times \$33/\text{wRVU} \times 50\% \text{ attribution} = \$45K$

We use Medicare's \$33 conversion factor and 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

WHAT THIS MEANS

wRVU improvement indicates that documentation is capturing more of the clinical complexity that was always present in your encounters. This isn't about changing how providers practice—it's about making sure the note reflects what actually happened in the room.

TIME IN NOTES

Documentation efficiency

6.0 min/encounter

saved per encounter

BEFORE ABRIDGE

8.00

min/encounter

AFTER ABRIDGE

2.00

min/encounter

=

+6.00

75% reduction

BENCHMARK

Typical Abridge customers see: 3-5 min reduction

You're at: 75% — Above typical

WHAT THIS MEANS

Each minute saved per encounter translates to 2,700 hours annually. This time can go toward patient care, work-life balance, or additional encounters—depending on your organizational priorities.

WORK OUTSIDE WORK

After-hours documentation

4.0 hrs/week

saved per encounter

BEFORE ABRIDGE

7.00

hrs/week

AFTER ABRIDGE

3.00

hrs/week

=

+4.00

57% reduction

BENCHMARK

Typical Abridge customers see: 2-4 hr/week reduction

You're at: 57% — Within typical range

WHAT THIS MEANS

Reducing after-hours documentation by 4.0 hours per week per provider directly improves work-life balance. This is often the most emotionally resonant metric for clinicians—it represents time reclaimed for family, rest, and personal pursuits.

SAME-DAY CHART CLOSURE

Documentation timeliness

BEFORE ABRIDGE

75.00

%

AFTER ABRIDGE

85.00

%

=

+10.00

10% lift

BENCHMARK

Typical Abridge customers see: 10-20 percentage point improvement

You're at: 10% — Within typical range

WHAT THIS MEANS

Same-day chart closure improves billing cycle times, reduces compliance risk, and indicates providers are completing documentation in real-time rather than batching. A +10% improvement suggests AI-assisted documentation is enabling more efficient workflows.

EARLY INDICATORS

WHAT'S WORKING WELL

- wRVU capture showing solid improvement (6% lift)
- Time in notes reduced by 6 minutes per encounter
- After-hours work reduced by 4.0 hours per week
- Same-day chart closure up 10 percentage points

AREAS TO WATCH

- ' Utilization at 60%—room to grow toward 80-85% at maturity
- ' Time savings not yet converting to patient access or overtime reduction

OPTIMIZATION OPPORTUNITIES

1. UTILIZATION FOCUS

+\$13K/yr

CURRENT

60%

TARGET

80-85%

GAP

20 pp

RECOMMENDED ACTION

Identify providers below 60% and address barriers to adoption

2. TIME CONVERSION

+\$61K/yr

CURRENT

2,700 hrs/year saved, 0% converted

TARGET

15-20% conversion to patient access

GAP

See action

RECOMMENDED ACTION

Review scheduling capacity with operations to convert saved time to visits

EARLY STAGE CONTEXT

At 60% utilization and 4 months in, you're still in early deployment. This is common—adoption takes time. Here's the path forward:

- Increase utilization to 70%: Value improves ~17%
- Increase utilization to 80%: Value improves ~33%
- Full adoption (85%): Unlocks maximum value potential

THE BOTTOM LINE

4 months in, you're building a foundation:

- \$45K in annual value from measurable improvements
- Efficiency gains captured but not yet converted to dollars
- Expansion potential of +\$148K at full scale

At 60% utilization, you're capturing less than half the potential. The path to multiplying this value is clear: push utilization to 80%+, validate gains, and consider converting efficiency to access.

THE PATH TO FULL SCALE

Based on your proven results at 30 providers and 60% utilization, here's what full-scale deployment could look like. These projections use your actual per-provider value generation—no hypotheticals.

\$192K

PROJECTED ANNUAL VALUE

At full scale

+\$148K

ADDITIONAL VALUE

Beyond current

90

TARGET PROVIDERS

75% utilization

SCALING MATH

Current annual value	\$45K
× Provider scale (30 '90)	3.0x
× Utilization improvement (60% '75%)	1.25x
Projected annual value at scale	\$192K

CURRENT VS. PROJECTED

CURRENT STATE		FULL SCALE TARGET	
Providers:	30	Providers:	90
Utilization:	60%	Utilization:	75%
Annual value:	\$45K	Annual value:	\$192K
Value/provider:	\$1K	Added value:	+\$148K

The value you're capturing today isn't a ceiling—it's a proof point. At 30 providers and 60% utilization, you've demonstrated \$45K in annual value. Scaling to 90 providers at 75% utilization projects to \$192K.

HOW WE CALCULATED YOUR VALUE

Every number in this report traces back to data you provided or industry-standard benchmarks. We've used conservative assumptions throughout—if anything, actual value is likely higher. Here's exactly how we calculated each component.

YOUR INPUTS

DEPLOYMENT

- 30 providers
- 45,000 annual encounters
- 60% utilization rate
- 4 months on Abridge

DOCUMENTED VOLUME

- 27,000 documented encounters
- = encounters × utilization × (months/12)

VALUE ATTRIBUTION

WRVU Value

We use Ware's \$33/wRVU conversion factor and apply 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

Time Conversion

Time savings are calculated from before/after documentation time. When converted to patient access, we use \$150/hour provider value and your specified conversion percentage.

Retention Value

Retention value costs estimated at \$500,000 per departure (recruitment, onboarding, lost productivity). Attribution to satisfaction improvement is conservative.

BENCHMARK RANGES

TYPICAL ABRIDGE RESULTS

- wRVU lift: 3-7%
- Time in notes reduction: 3-5 min/encounter
- Chart closure improvement: 5-15 pp

QUALITY OF LIFE METRICS

- Pajama time reduction: 2-5 hrs/week
- Satisfaction improvement: 10-20 points
- Burnout reduction: varies by baseline

WHAT WE DON'T INCLUDE

- Downstream revenue from improved patient experience and retention
- Quality measure improvements (MIPS, HEDIS) and associated incentives
- Reduced compliance and audit risk from better documentation
- Training and onboarding time reduction for new providers
- Long-term career satisfaction and reduced early retirement

Questions about our methodology? We're happy to walk through any calculation in detail. The goal is confidence in every number—if something doesn't make sense, we want to know.